

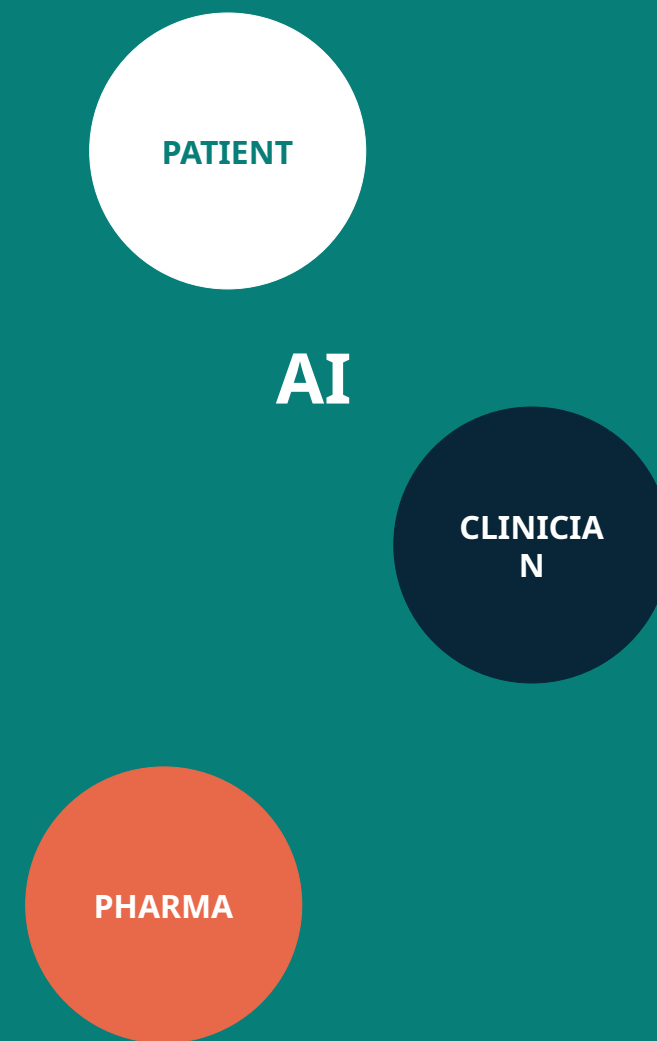
PATIENT-CENTRIC HEALTHCARE 5.0

AI for Better Conversations, Safer Decisions,
Professional Impact and Research

60 MIN THEORY + 90 MIN WORKSHOP

Doctors × Pharma Professionals

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Goa Institute of Management



The first question is not “What can AI do?”

It is:

Does this give the patient more attention, understanding and safety?

PAIR DISCUSSION

Where does technology currently pull attention away from the patient—or delay a safe decision?

Design principle: automate friction, preserve relationship and make ownership visible.

Four capabilities—not a catalogue of apps

OWN**Own projection**

Build trusted professional visibility without promotion or unsupported claims.

RESEARCH**Find and appraise**

Move from a focused question to traceable evidence and limitations.

DIAGNOSIS**Support reasoning**

Expose hypotheses, urgency and missing information—never outsource diagnosis.

PATIENT**Improve continuity**

Return attention, clarity, teach-back and follow-up to the care journey.

Use P-A-T-I-E-N-T before any healthcare AI workflow



Protect privacy



Assess accuracy



Trace sources



Inform + consent



Examine equity



Never replace
judgement



Take responsibility

THE 20-SECOND PAUSE

Could I explain the input, source, uncertainty and final decision to this patient—or to a regulator?

Seven cases. One operating question.

Where does AI return attention or insight—and where must human ownership become stronger?

AI assists • Professionals decide • Patients remain the purpose

Serbia EHR: EHR-embedded X-ray triage and no-show prediction

2025 OPERATIONAL CASE STUDY AND LOCAL MODEL VALIDATION

Health Center Nis; MEDIS.NET across 25 facilities

AI + workflow

VGG19-based chest X-ray classification plus appointment-attendance prediction embedded in the EHR

Reported result

The article reports about 92% X-ray classification accuracy and a substantial reduction in no-shows when models and staff action were combined.

EVIDENCE LIMIT

Single health-system context; model performance and workflow results require external and local validation.

HUMAN CHECKPOINT

Radiologist signs the report; staff decide outreach or scheduling action.

Embed probability in the workflow; do not hide decision ownership.

Which decision should remain visibly human even if the prediction is accurate?

Serbia lesson: prediction works only when the workflow owns the next step

MODEL**Probability**

Accuracy belongs to a defined task and validation population.

WORKFLOW**Action**

Staff decide outreach; radiologists retain reporting responsibility.

MEASURE**Patient effect**

Track access, false alerts, missed cases, equity and burden—not only AUC.

SepsisLab: Why clinicians ignore alerts—and how design can restore trust

2024 QUALITATIVE STUDY AND HUMAN-AI PROTOTYPE EVALUATION

Six experienced US clinicians discussing the Epic Sepsis Model and a prototype

AI + workflow

Trend and uncertainty visualisation, ranked information needs and what-if simulation

Reported result

Clinicians described the redesigned system more as a collaborator than a static score.

EVIDENCE LIMIT

Very small qualitative sample and prototype setting; not an outcome or safety trial.

HUMAN CHECKPOINT

Clinician interprets risk, orders information and decides action.

Timely, explainable and actionable support can beat another interruptive alert.

Does this alert arrive before the decision—and tell us what could reduce uncertainty?

Redesign the alert as a conversation with uncertainty

STATIC SCORE

- Interrupts late
- No trend or uncertainty
- Weak next-information guidance
- Override becomes reflex

COLLABORATIVE SUPPORT

- Shows what changed
- Surfaces uncertainty
- Suggests information to reduce it
- Records review and override

Design for the decision before it happens—not an explanation after it.

Trust and adoption

A model enters a social system: consent, accountability, review workload and patient experience determine use.

AI assists • Professionals decide • Patients remain the purpose

Qatar perceptions: What family physicians expect and fear

2024 CROSS-SECTIONAL PERCEPTION SURVEY

Qatar Primary Health Care Corporation; 102 eligible family-physician responses

AI + workflow

Survey of perceived clinical, administrative and public-health applications

Reported result

78.4% reported AI awareness; respondents perceived value in task management but raised confidentiality, accountability and patient-satisfaction concerns.

EVIDENCE LIMIT

Self-reported perceptions from 102 of 724 invited physicians; not a clinical-effectiveness study.

HUMAN CHECKPOINT

Physicians asked for training, governance and clear accountability.

Adoption is a trust and operating-model problem, not only a model problem.

What assurance would your colleagues need before using a new AI tool?

Ambient scribes: The real workflow cost and payoff of ambient documentation

IMPLEMENTATION DISSERTATION PLUS GROWING HEALTH-SYSTEM OBSERVATIONAL EVIDENCE

Five-practice deployment described in a DBA dissertation, triangulated with multi-site peer-reviewed ambient-scribe evidence

AI + workflow

Conversation capture with consent and a draft note for clinician review

Reported result

Reports describe initial resistance, new review work and potential reductions in after-hours EHR time and burden after adoption.

EVIDENCE LIMIT

Product, setting, adoption maturity and outcome measures vary; product-specific safety and local effect require validation.

HUMAN CHECKPOINT

Clinician or designated reviewer edits and signs; organisation owns consent, retention and audit.

Time saved at the keyboard creates a new quality-assurance workflow.

Who reviews the reviewer—and what error would stop the pilot?

Adoption is an operating-model decision

NOTICE

What does the patient know and choose?

OWNERSHIP

Who edits, signs, escalates and audits?

WORKLOAD

What new review work appears?

MEASURES

Attention, accuracy, burden, equity and experience

STOP RULE

What error or harm pauses the pilot?

Learning and future readiness

AI can accelerate preparation and make futures vivid; faculty and professionals must protect reasoning, originality and values.

AI assists • Professionals decide • Patients remain the purpose

China teaching: Generative AI in case-based medical learning

2025 COMPARATIVE EDUCATIONAL STUDY

Chengdu Medical College; 79 students, control n=40 and AI group n=39

AI + workflow

Kimi Chat 2.0 used to research and present a liver-cancer case

Reported result

Assignment time was 2.6 vs 5.5 hours; final scores 77.3±4.3 vs 66.5±5.4; teacher questions fell from 35 to 10.

EVIDENCE LIMIT

Two intact classes in one course; not randomised across institutions; learning and grading findings should not be treated as clinical outcomes.

HUMAN CHECKPOINT

Faculty discussion remains essential for reasoning, originality and challenge.

AI raises the floor and compresses search time; it can also homogenise thinking.

How do we save time without reducing productive struggle?

Save time without deleting productive struggle

BEFORE**Commit first**

Learner writes a problem representation and uncertainty before asking AI.

DURING**Challenge**

AI asks for evidence, contradictions, red flags and revision—not instant answers.

AFTER**Audit**

Faculty checks source use, reasoning change, originality and unsupported claims.

Trinity foresight: AI-supported scenario planning for medical curricula

2025 SINGLE-INSTITUTION QUALITATIVE PARTICIPATORY-DESIGN CASE

Trinity College Dublin; one workshop with 28 participants

AI + workflow

ChatGPT narratives, MidJourney visuals and Murf.AI narration for four 2035 scenarios

Reported result

Participants converged on systems thinking, patient-centredness, continuous learning, participatory health, sustainability and technological adaptability.

EVIDENCE LIMIT

One workshop, no patient representatives, possible groupthink and framing effects; not universally generalisable.

HUMAN CHECKPOINT

Facilitators reviewed and refined all AI outputs; participants chose competencies and actions.

AI can make futures tangible; people must choose values and priorities.

Which capability remains essential in every plausible future?

Make futures tangible; keep values human

SYSTEMS THINKING

PATIENT-CENTREDNESS

CONTINUOUS LEARNING

PARTICIPATORY HEALTH

SUSTAINABILITY

TECH ADAPTABILITY

AI can generate scenarios. People choose the capabilities and trade-offs worth preserving.

Explainability and due diligence

A visible explanation can support scrutiny; it does not prove correctness, benefit or fairness.

AI assists • Professionals decide • Patients remain the purpose

Explainable AI: Why a visible explanation is necessary—but not sufficient

NARRATIVE REFERENCE CHAPTER WITH ILLUSTRATIVE CASE APPLICATIONS

2025 reference chapter covering imaging, drug discovery and personalised medicine

AI + workflow

Heatmaps, attention maps and feature attribution to expose model influences

Reported result

The chapter illustrates how explanation can support audit, teaching and clinician sense-checking.

EVIDENCE LIMIT

An explanation can be plausible yet unstable or misleading; explainability is not proof of accuracy or benefit.

HUMAN CHECKPOINT

Professional compares the explanation with intended use, validation and patient context.

Ask why, for whom and under what validation—not only what the model predicted.

Is the explanation clinically meaningful or merely decorative?

Eight questions turn a demo into due diligence

01

Intended use + user?

02

**Target population +
setting?**

03

**Comparator +
outcomes?**

04

**False negatives +
positives?**

05

Subgroup performance?

06

**External/local
validation?**

07

Override + audit trail?

08

**India status + data
controls?**

A persuasive heatmap is not an outcome study.

Seven cases, seven evidence labels

CASE	DESIGN	USEFUL SIGNAL	DO NOT CLAIM
Serbia	Operational case	Workflow integration	Universal performance
Qatar	Survey	Trust barriers	Clinical effectiveness
China	Education comparison	Time/score signal	Clinical benefit
Trinity	Participatory case	Future capabilities	Generalisability
SepsisLab	6-clinician prototype	Design insight	Outcome/safety benefit
Scribes	Implementation evidence	Workflow trade-off	Product-wide safety
XAI	Narrative chapter	Audit questions	Accuracy from explanation

India guardrails: legal compliance is the floor; professional accountability is higher

DATA**DPDP compliance**

Purpose, notice/consent where applicable, minimisation, security, retention and rights.

CLINICAL**ICMR ethics**

Safety, autonomy, transparency, accountability, equity and human oversight.

DEVICE**Product status**

Verify intended use and CDSCO status for medical-device software; do not infer approval.

PHARMA**UCPMP + SOPs**

Non-promotional medical information; immediate PV/PQC escalation; approved review.

Theory closes with one decision rule

Use AI when it strengthens

attention • understanding • evidence • continuity

and stop when it obscures

ownership • uncertainty • privacy • urgent escalation

60 minutes complete → open the workshop workbook

Hands-on workshop

Four stations: consultation continuity, diagnostic support, patient/pharma communication, and evidence/projection.

AI assists • Professionals decide • Patients remain the purpose

Access determines demo mode—not learning value

LIVE**Free / free-tier**

ChatGPT, Heidi*, Medscape AI*, DxGPT, MDCalc, PubMed, ResearchRabbit.

*Account/region may apply.

FACILITATOR**Paid / trial**

Nabla, VisualDx, Epocrates premium, Elicit/Scite paid features, Canva/Gamma premium.

SHOWCASE**Institutional**

Abridge, ClinicalKey AI, Lexicomp/Micromedex, Veeva and enterprise PV/MI platforms.

If access fails: use prepared screenshots/output → analyse with ChatGPT → score with the same clinical checkpoint.

Paste this wrapper before every medical discussion prompt

You are supporting a synthetic healthcare education exercise. You are not the treating clinician and must not replace professional judgement. Do not provide a final diagnosis, prescribe, change a dose, provide reassurance or instruct anyone to delay urgent care. Separate supplied facts, patient-reported information, professional observations, assumptions, unknowns and AI-generated hypotheses. Do not invent findings, medicines, laboratory results, citations or guidelines. Identify red flags and information requiring urgent professional review. Use only the information and approved sources supplied. End with a section titled 'Human decision and verification required'.

Synthetic data only • No final diagnosis or prescribing • Human verification

Station 1: Consultation continuity

Compare an ambient-scribe workflow with a structured ChatGPT draft—then find what must be corrected.

AI assists • Professionals decide • Patients remain the purpose

Three tools, one governed task

LIVE

Heidi Health

Free tier + paid; synthetic transcript only.
<https://scribe.heidihealth.com>

TRIAL

Nabla

Trial/enterprise; compare note structure and review controls. <https://app.nabla.com>

SHOW

Abridge

Enterprise demo/product walkthrough.
<https://www.abridge.com/product>

EVERYONE

ChatGPT

Use Prompt 1 for tagged facts, unknowns, teach-back and escalation.

Synthetic consultation: continuity, not diagnosis

SYNTHETIC

Mrs A, 52

Type 2 diabetes + hypertension
Missed medicines twice this week
Low health literacy; prefers Hindi
Tingling feet; no acute red flags supplied
Believes insulin means “failure”

CREATE

- 1 Reviewable draft summary
- 2 Facts / unknowns / hypotheses tags
- 3 Medication-reconciliation questions
- 4 Patient summary + teach-back
- 5 Follow-up and escalation items

Specialised-tool demo: capture → draft → review → release

1 NOTICE

Synthetic
encounter;
consent script

2 CAPTURE

Enter prepared
transcript; no
recording needed

3 DRAFT

Generate note +
patient
instructions

4 AUDIT

Find omissions,
inventions and
unsafe phrasing

5 DECIDE

Edit, sign or reject;
record reason

Stop rule: an invented diagnosis, dose, allergy or red-flag disposition enters the draft unnoticed.

ChatGPT fallback: create a tagged, reviewable draft

PROMPT 1

Using the synthetic consultation transcript below, create: (1) a source-grounded draft encounter summary, (2) a problem-information list, (3) medication-reconciliation questions, (4) a plain-language patient summary, (5) three teach-back questions, and (6) follow-up and escalation items. Tag every statement [PATIENT REPORTED], [PROFESSIONAL OBSERVATION], [UNKNOWN] or [NEEDS VERIFICATION]. Do not invent examination findings, diagnosis, medication, dose, investigation, reassurance or follow-up decision. Transcript: [PASTE SYNTHETIC TRANSCRIPT]

EXPECTED OUTPUT

Tagged draft note, patient summary, questions and escalation items

HUMAN CHECKPOINT

Reconcile facts and medicines; check red flags, consent, local language and record policy before sign-off.

Debrief: what attention returned—and what review work appeared?

RETURNED**Attention**

Less screen switching; faster draft; consistent teach-back structure.

CREATED**Verification**

Omissions, hallucinations, tone, medicine reconciliation, consent and release review.

DECISION**Pilot rule**

Measure time, error taxonomy, patient experience, after-hours work and stop criteria.

Score both tools with the same outcome and safety criteria.

Station 2: Diagnostic support + medication safety

Use purpose-built tools to widen attention, then triangulate with evidence, calculators and drug references.

AI assists • Professionals decide • Patients remain the purpose

Match the tool to the clinical question

EVIDENCE

Medscape AI / ClinicalKey AI

Cited orientation; free registered vs institutional access.

DIFFERENTIAL

DxGPT / VisualDx / iatroX

Hypotheses, missing information and visual context; scope varies.

CALCULATE

MDCalc

Bounded calculators with source studies; verify every input.

MEDICINES

Epocrates / institutional references

Drug facts and interactions; confirm India formulary and local protocol.

Synthetic case: make uncertainty visible

SYNTHETIC

Mr R, 58

Progressive breathlessness + ankle swelling
BP 164/96; pulse 104; SpO₂ 93%
Diabetes, hypertension, smoker
Raised BNP reported; basal crepitations
Medication and ECG information incomplete

TEAM QUESTION

Which information changes urgency before it changes the differential?

Common/likely
Dangerous must-not-miss
Alternatives
Missing evidence
Human disposition decision

ChatGPT exercise: reason without outsourcing diagnosis

PROMPT 2

Using only the synthetic case below: (1) write a one-sentence problem representation without declaring a diagnosis; (2) group hypotheses as common/likely, dangerous must-not-miss, and alternative explanations; (3) for each, list supporting information, contradictory information, missing information, red flags, time sensitivity and evidence that must be verified; (4) identify the five questions or observations most likely to change urgency; and (5) state what a licensed clinician must decide. Do not recommend treatment or invent probabilities. Case: [PASTE SYNTHETIC CASE]

EXPECTED OUTPUT

Structured hypotheses and uncertainty map

HUMAN CHECKPOINT

Check urgent red flags, primary evidence, local protocol and patient context; clinician owns diagnosis and disposition.

Purpose-built comparison: same question, different constraints

TOOL	ACCESS	DEMO TASK	HUMAN CHECK
DxGPT	Free	Differential + gaps	Check omissions, urgency, intended use
Medscape AI	Free registered	Evidence orientation	Open cited source; population + date
VisualDx	Paid/trial	Visual differential	Never upload an identifiable image
ClinicalKey AI	Institutional	Curated corpus	Verify licensed content + local protocol

Compare coverage and uncertainty—not which interface looks most confident.

Medication reconciliation is a verification workflow

PROMPT 3 OUTPUT

Confirmed facts
Potential issues to verify
Missing name/strength/route/frequency
Patient and prescriber/pharmacist questions
Approved sources to check

DO NOT ASK CHATGPT TO

Start, stop or change a medicine
Invent an interaction or allergy
Silently calculate with missing inputs
Substitute for pharmacist/prescriber review
Override local formulary or protocol

Full copy-ready medication prompt: Prompt Card 3

Triangulate: AI output × evidence × local context



Accept the supported overlap—and document the remaining uncertainty.

Debrief: did the tools widen attention or create noise?

- 1 What urgent feature was missed?

- 2 Which claim lacked source support?

- 3 Which input changed a score?

- 4 What local protocol controlled the action?

- 5 What remained a licensed-clinician decision?

Station 3: Patient + pharma communication

Create an evidence-bounded patient handout, triage a medical-information interaction and build ethical professional visibility.

AI assists • Professionals decide • Patients remain the purpose

Communication stack: content, design, language and governance

CONTENT

ChatGPT

Draft from approved evidence; mark unsupported claims and reviewer gates.

DESIGN

Canva / Gamma

Free-tier + paid; convert approved content into an accessible visual asset.

LANGUAGE

Bhashini

India language enablement; clinical meaning still needs bilingual review.

PHARMA

Approved MI/PV systems

Institutional SOPs control medical response, adverse events and product quality.

Build a patient handout from approved evidence only

PROMPT 4

Using only the approved source excerpt below, draft a one-page patient handout for [AUDIENCE] at class 6–8 reading level. Include a short title, five practical actions, medicine-taking reminders without products or doses, monitoring, routine follow-up, three urgent red flags, three teach-back questions, source, review date, reviewer placeholder and AI-assistance disclosure. Mark unsupported statements [REVIEW]. Provide a draft Hindi version, marking medical terms for bilingual professional review. Do not invent statistics, guarantees or clinical advice outside the source. Source: [PASTE APPROVED SOURCE]

EXPECTED OUTPUT

Plain-language handout with red flags and teach-back

HUMAN CHECKPOINT

Clinical meaning, translation, accessibility, source, version and reviewer approval.

Pharma exercise: medical information + safety triage

PROMPT 7

Analyse the synthetic medical-information interaction. Create: the exact unsolicited scientific question; minimum information needed; approved sources; a neutral response outline; statements requiring medical/legal/regulatory review; possible off-label elements requiring medical-information handling; possible adverse-event or product-quality information requiring immediate escalation; follow-up commitments; and documentation requirements. Do not create promotional claims, assess causality, minimise an event or delay escalation. Interaction: [PASTE SYNTHETIC INTERACTION]

EXPECTED OUTPUT

Neutral inquiry plan plus explicit safety/PQC escalation flags

HUMAN CHECKPOINT

Medical information, pharmacovigilance and MLR follow the approved SOP; safety escalation is immediate.

Own projection: build trust without crossing into promotion

PROMPT 10

Using only the approved evidence below, draft a 150–200 word professional post for [DOCTOR / MEDICAL-AFFAIRS PROFESSIONAL] to [EDUCATE PATIENTS / EDUCATE PEERS / DISCUSS AN EVIDENCE GAP]. Include one evidence-based message, one practical implication, one limitation, a visible source, AI-assistance disclosure and an invitation to professional discussion—not individual treatment advice. Do not add product promotion, off-label claims, exaggerated certainty or authority beyond the stated role. Evidence: [PASTE]

OUTPUT

Concise, sourced and bounded professional post

RELEASE GATE

Clinical/MLR review, citation, disclosure, version owner and publication approval.

One review gate prevents three classes of harm

CLINICAL

Meaning

Accurate, actionable, red flags separated, teach-back possible.

REGULATORY

Boundary

No promotion, unsupported claim, off-label drift or delayed PV/PQC escalation.

ACCESS

Language + format

Readable, culturally appropriate, bilingual-reviewed and accessible.

Nothing reaches a patient or external audience until the named reviewer approves it.

Station 4: Research + teaching

Use a transparent evidence stack, then turn it into a Socratic discussion—not an answer generator.

AI assists • Professionals decide • Patients remain the purpose

A research stack is a sequence of checks

DISCOVER

PubMed + Elicit

Free search plus free-tier synthesis; inspect query, inclusion and source records.

MAP

ResearchRabbit

Free core; follow citation and author networks without confusing proximity with quality.

CHECK

Scite

Limited free + paid; inspect citation context, then open the original study.

DISCUSS

ChatGPT

Structure PICO, appraisal, uncertainty and Socratic learning using supplied sources.

Start with a decision-shaped question

P Population

Which patients or professionals?

I Intervention

What AI-enabled workflow?

C Comparator

Usual care or alternative?

O Outcome

Patient, safety, time, equity or cost?

Example: ambient documentation versus usual documentation → after-hours EHR time + note errors + patient experience

ChatGPT exercise: rapid evidence map with transparent limits

PROMPT 11

Convert the question into a transparent rapid-research workflow. Produce PICO; concepts and synonyms; a draft PubMed Boolean query; inclusion and exclusion criteria; evidence-extraction fields; citation-verification checklist; bias/applicability questions; uncertainty log; and a statement of what this rapid review cannot conclude. Do not invent citations. Label every citation [UNVERIFIED] until located in PubMed, a registry or the original publication. Question: [PASTE]

EXPECTED OUTPUT

Search strategy, extraction frame and uncertainty log

HUMAN CHECKPOINT

Every citation and extracted field is verified in the original source; methods and limitations are documented.

Teaching exercise: ask, challenge, revise—then answer

PROMPT 9

Act as a Socratic facilitator for the synthetic case. Do not reveal the answer immediately. Ask the learner to create a problem representation; propose common, dangerous and alternative hypotheses; identify evidence for/against; identify missing information and urgency; challenge one assumption; introduce one verified source; ask the learner to revise; then provide feedback with a transparent rubric and identify unsupported AI-generated information. Case: [PASTE]

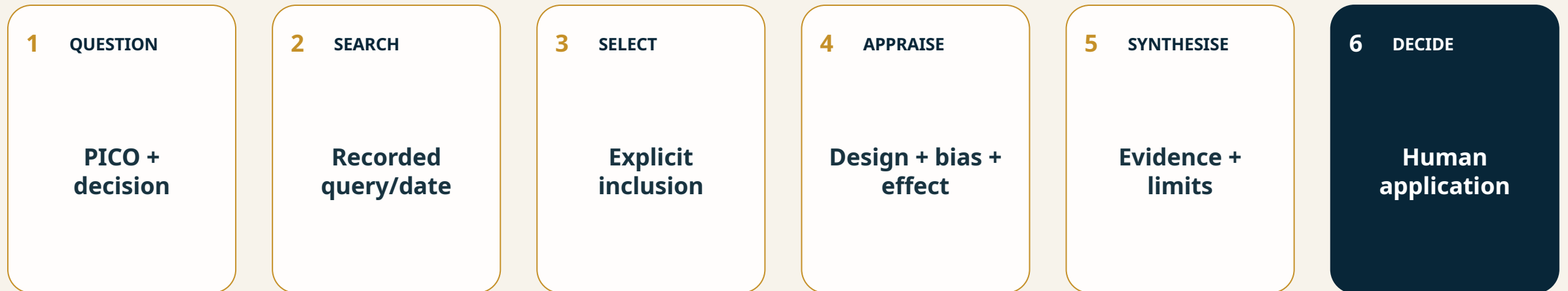
EXPECTED OUTPUT

Staged questions, revised reasoning and rubric feedback

HUMAN CHECKPOINT

Faculty verifies content and preserves learner effort, dialogue and originality.

Research integrity is a chain of custody



Stop if a citation cannot be opened, the design is mislabelled, or an AI summary outruns the source.

Use one scorecard for every tool

P**PATIENT VALUE**

Attention, understanding, access or safety

E**EVIDENCE**

Original sources, validation and limits

O**OWNERSHIP**

Named reviewer, release and escalation

P**PRIVACY**

Minimum data, consent, retention and location

L**LOCAL FIT**

India access, language, protocol and workflow

E**EFFORT**

Training, review burden, cost and integration

Your 30-day capstone is a controlled experiment

- 1** **USE CASE** One friction point, one user, one patient value
- 2** **BASELINE** Time, errors, experience or access before AI
- 3** **PILOT** Small scope, synthetic/test data first, named owner
- 4** **CHECK** **PATIENT + POPLE** scorecard, sampling and incident path
- 5** **DECIDE** Continue, redesign or stop on a named date

Patient centricity is the outcome—not the slogan

The best AI workflow makes the next human conversation

CLEARER

SAFER

MORE HUMAN

AI assists. Professionals decide. Patients remain the purpose.

90 minutes complete • total programme = 150 minutes

Specialised-tool menu for facilitator selection

AMBIENT**Heidi • Nabla • Abridge****EVIDENCE****Medscape AI • ClinicalKey AI****REASONING****DxGPT • VisualDx • iatroX****MEDICINES****MDCalc • Epocrates • Lexicomp •
Micromedex****COMMUNICATION****Canva • Gamma • Bhashini****RESEARCH****PubMed • Elicit • ResearchRabbit •
Scite**

Copy-ready prompt index

P1	Consultation continuity	ChatGPT; compare with Heidi or Nabla	P7	Pharma medical-information and safety triage	ChatGPT discussion prompt; compare with approved medical-information/PV workflow
P2	Diagnostic reasoning without outsourcing diagnosis	ChatGPT; compare with DxGPT and a clinical evidence tool	P8	Explainable-AI vendor evaluation	ChatGPT discussion prompt
P3	Medication-reconciliation discussion	ChatGPT; compare with Epocrates, Lexicomp, Micromedex or an approved reference	P9	Socratic medical teaching	ChatGPT
P4	Patient-education asset	ChatGPT plus Canva/Gamma; bilingual review may use Bhashini	P10	Professional projection	ChatGPT plus approved publishing/design workflow
P5	Redesigning an unhelpful clinical alert	ChatGPT discussion prompt	P11	Rapid evidence map	ChatGPT plus PubMed/Elicit/ResearchRabbit/Scite
P6	Ambient-scribe pilot plan	ChatGPT; compare with Heidi/Nabla governance settings			

India governance sources to open before procurement

MEITY

Digital Personal Data Protection Act/Rules<https://www.meity.gov.in/>

ICMR

Ethical Guidelines for AI in Biomedical Research and Healthcare<https://ethics.ncdirindia.org/>

CDSCO

Medical device/software guidance and notices<https://cdsco.gov.in/>

DEPARTMENT OF PHARMACEUTICALS

UCPMP 2024<https://pharmaceuticals.gov.in/>

NMC

Professional and telemedicine guidance<https://www.nmc.org.in/>

Seven case sources

CASE 1	EHR-embedded X-ray triage and no-show prediction	https://www.mdpi.com/2073-431X/14/8/320
CASE 2	What family physicians expect and fear	https://ai.jmir.org/2024/1/e40781
CASE 3	Generative AI in case-based medical learning	https://link.springer.com/article/10.1186/s12909-025-07567-z
CASE 4	AI-supported scenario planning for medical curricula	https://www.jmir.org/2025/1/e73173
CASE 5	Why clinicians ignore alerts—and how design can restore trust	https://pmc.ncbi.nlm.nih.gov/articles/PMC11149368/
CASE 6	The real workflow cost and payoff of ambient documentation	https://www.proquest.com/openview/574eebe7a1f1dcd2ca5d88f6a9a22a7b/1?pq-origsite=gscholar&cbl=18750&diss=y
CASE 7	Why a visible explanation is necessary—but not sufficient	https://www.sciencedirect.com/science/article/pii/B9780443239793000063

Demo failure is a facilitation branch—not a workshop failure

FAILURE	FALLBACK	LEARNING PRESERVED
No account	Prepared screenshots/output	Analyse output with same prompt + checkpoint
Network failure	Offline synthetic packet	Role-play reviewer and patient advocate
Tool region-blocked	ChatGPT paired prompt	Compare to saved specialised-tool output
Model unsafe output	Stop and label failure	Apply escalation + pilot stop rule
Time overrun	Skip interface tour	Keep case, decision and debrief
Privacy concern	Do not enter data	Use only the supplied synthetic vignette